



1. 12 months calendar

WHAT YOU SEE

'12 MONTHS' wall calendar top-left

WHAT IT ENCODES

After ACS (including STEMI) with stenting, the default duration of dual antiplatelet therapy (DAPT) is ~12 months: aspirin 81 mg daily lifelong PLUS a P2Y12 inhibitor for 12 months. Preferred P2Y12 agents after ACS are ticagrelor (90 mg BID) or prasugrel (10 mg daily); clopidogrel (75 mg daily after a 300–600 mg load) is the alternative when potent agents are contraindicated. Duration can be shortened (to 1–6 months) if high bleeding risk, or extended if high ischemic/low bleeding risk.

BOARD TRAP

WRONG: DAPT for only 1 month after ACS. Discriminator: 1 month is a short course reserved for high-bleeding-risk patients with a modern DES — the standard after ACS is 12 months; calling 1 month the default is the trap.

2. NOT DAPT bottle (lone aspirin)

WHAT YOU SEE

Crossed-out pill bottle 'NOT DAPT'

WHAT IT ENCODES

A single antiplatelet agent alone is inadequate in the early period after stenting — aspirin monotherapy does not prevent stent thrombosis while the struts are re-endothelializing. You need DUAL therapy (aspirin + P2Y12 inhibitor). Only after the DAPT period (≥12 months post-ACS) do you de-escalate to a single agent for life.

BOARD TRAP

WRONG: aspirin monotherapy suffices after stenting. Discriminator: in the vulnerable window aspirin alone leaves the patient unprotected against stent thrombosis — dual therapy is mandatory until endothelialization is complete.

3. Thrombosis bench

WHAT YOU SEE

Lower-left bench 'THROMBOSIS'

WHAT IT ENCODES

Premature discontinuation of DAPT is the strongest precipitant of acute stent thrombosis — an abrupt, catastrophic thrombotic occlusion of the stented segment presenting as a new STEMI/sudden death. This is why patients must NOT stop DAPT early (e.g., for elective surgery) without cardiology input; elective non-cardiac surgery should be deferred until the minimum DAPT period is met.

BOARD TRAP

WRONG: stent thrombosis risk is unrelated to DAPT. Discriminator: stopping DAPT early is the single biggest modifiable cause of stent thrombosis — the link is causal and heavily tested in perioperative-management questions.

4. 3–9x death/MI risk

WHAT YOU SEE

Knight '3–9x', 'DEATH 10–40%', 'MI 30–70%'

WHAT IT ENCODES

Stent thrombosis is highly lethal: it raises the risk of death/MI several-fold (~3–9x), with mortality roughly 10–40% and a very high rate of MI (30–70%) when it occurs. It is an emergency requiring immediate repeat angiography/PCI. Contrast this acute, dramatic event with restenosis, which is gradual and far less lethal.

BOARD TRAP

WRONG: stent thrombosis is usually benign. Discriminator: stent thrombosis is a medical emergency with high mortality (unlike restenosis) — treating it as a minor event is the error; it presents as acute MI.

5. Coronary anatomy first horse

WHAT YOU SEE

Horses + fence 'CORONARY ANATOMY FIRST'

WHAT IT ENCODES

Before committing to PCI, define the full coronary anatomy and disease burden — this determines whether PCI or CABG is the better revascularization. CABG is generally favored for left main disease, complex multivessel disease (high SYNTAX score), and diabetics with multivessel disease; PCI suits focal/single-vessel disease and the acute STEMI culprit. Heart-team discussion is standard for complex anatomy.

BOARD TRAP

WRONG: stent immediately without assessing the full anatomy. Discriminator: in complex/left-main/diabetic multivessel disease, CABG can be superior — reflexively stenting without weighing the anatomy (SYNTAX) misses the PCI-vs-CABG decision.

6. Stroke-brain (load only ≤?)

WHAT YOU SEE

Horse + 'stroke-brain' note

WHAT IT ENCODES

Choose the P2Y12 agent by bleeding/stroke risk. Prasugrel is CONTRAINDICATED in any prior stroke/TIA (net harm from intracranial bleeding) and is avoided in age ≥75 or weight <60 kg. Ticagrelor is the potent alternative there (but causes dyspnea and requires BID dosing and low-dose aspirin). Clopidogrel is the gentlest option for the highest bleeding-risk patients.

BOARD TRAP

WRONG: load every patient with the same potent P2Y12 inhibitor regardless of stroke risk. Discriminator: prasugrel is absolutely contraindicated with prior stroke/TIA — picking it for a patient with cerebrovascular history is a classic harm trap.

7. BVS / bioresorbable (crossed)

WHAT YOU SEE

Green scaffold panel '20–50% RESTENOSIS'

WHAT IT ENCODES

Older bare scaffolds / balloon angioplasty alone produce high restenosis (~20–50%) from elastic recoil and neointimal hyperplasia. Bioresorbable vascular scaffolds (BVS) were withdrawn from routine use after showing higher rates of scaffold thrombosis and target-lesion failure than modern DES. They are not first-line.

BOARD TRAP

WRONG: bioresorbable scaffolds are first-line. Discriminator: BVS were largely abandoned for higher thrombosis/failure — current first-line is a second-generation drug-eluting stent, not a bioresorbable scaffold.

8. BMS 10–30% restenosis

WHAT YOU SEE

'BMS' panel '10–30% RESTENOSIS'

WHAT IT ENCODES

Bare-metal stents have higher restenosis (~10–30%) than DES but historically allowed a SHORTER mandatory DAPT duration because they re-endothelialize faster. Their main niche today is patients who cannot tolerate prolonged DAPT (imminent surgery, very high bleeding risk), though modern DES now also permit short DAPT, narrowing this advantage.

BOARD TRAP

WRONG: BMS has lower restenosis than DES. Discriminator: BMS has MORE restenosis than DES — its only relative edge was shorter required DAPT, not better patency.

9. DES 5–15% restenosis

WHAT YOU SEE

'DES' panel '5–15% RESTENOSIS'

WHAT IT ENCODES

Drug-eluting stents elute an antiproliferative agent (e.g., everolimus, zotarolimus, sirolimus) that suppresses neointimal hyperplasia, giving the lowest restenosis rate (~5–15%). They are the preferred stent for nearly all PCI today. Second-generation DES also have low thrombosis rates and now permit shortened DAPT when bleeding risk is high.

BOARD TRAP

WRONG: DES has the highest restenosis rate. Discriminator: DES has the LOWEST restenosis (the drug blocks neointimal growth) — it is preferred precisely for that; claiming the opposite reverses the whole rationale.

10. DES late thrombosis clock

WHAT YOU SEE

'MORE LATE THROMBOSIS' clock by DES

WHAT IT ENCODES

First-generation DES delayed endothelialization, raising LATE/very-late stent thrombosis risk — which is why prolonged DAPT became important with DES. Second-generation DES (everolimus/zotarolimus) markedly reduced this, so late thrombosis with modern stents is now low and DAPT can sometimes be shortened.

BOARD TRAP

WRONG: DES never cause late thrombosis. Discriminator: the delayed-healing of (especially first-gen) DES is exactly why late stent thrombosis was a concern and DAPT was prolonged — denying any late thrombosis risk is the trap.

11. Restenosis bench

WHAT YOU SEE

Right bench 'RESTENOSIS'

WHAT IT ENCODES

Restenosis is gradual re-narrowing of the stented segment from neointimal hyperplasia (smooth-muscle proliferation), evolving over months. It is a distinct process from stent thrombosis: restenosis is a slow re-stenosis presenting as recurrent stable angina, whereas thrombosis is an abrupt clot causing acute MI. DAPT prevents thrombosis, not restenosis.

BOARD TRAP

WRONG: restenosis and stent thrombosis are the same process. Discriminator: restenosis = gradual neointimal regrowth (recurrent angina); thrombosis = acute clot (STEMI) — different mechanism, timeline, and lethality.

12. Gradual 12 months sign

WHAT YOU SEE

'12 MONTHS — GRADUAL' anvil sign

WHAT IT ENCODES

Restenosis typically presents GRADUALLY over months (commonly 3–12 months post-PCI) as recurrent exertional/stable angina, not as sudden collapse. The slow tempo (vs the abrupt presentation of stent thrombosis) is the clinical discriminator on exam questions.

BOARD TRAP

WRONG: restenosis presents as sudden total occlusion / acute MI. Discriminator: a gradual return of angina over months = restenosis; a sudden STEMI = stent thrombosis — matching the tempo to the diagnosis is the tested point.